TEAM Model

Transforming Episode Accountability Model

Market Opportunity, Competitive Landscape
& Product Strategy

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Executive Summary

The CMS Transforming Episode Accountability Model (TEAM) represents one of the most significant payment reforms in Medicare history. Beginning January 1, 2026 and running through December 2030, TEAM mandates that approximately 740 acute care hospitals in selected Core-Based Statistical Areas assume financial accountability for the total cost and quality of five major surgical episodes, spanning from surgery through 30 days post-discharge.

This report analyzes the market opportunity created by TEAM, maps the competitive landscape of existing vendors, identifies seven explicit gaps where no current solution adequately serves hospitals, and proposes a product strategy to address the three most compelling gaps. We also outline a go-to-market model, persona-specific value propositions, and a phased product roadmap.

Key Findings

Market size: ~740 mandatory hospitals, 5-year model, billions in episode spending at stake

Competitive gap: No vendor provides a real-time, hospital-side episode command center with TEAM-economics-aware discharge decision support

Recommended play: Build a 3-module SaaS product combining episode cost tracking, discharge optimization, and financial simulation

GTM wedge: Enter via CFO/VP Finance with the financial simulator, expand to care management with the operational modules

1. The TEAM Model: What Hospitals Face

TEAM is a mandatory, episode-based alternative payment model created by the CMS Innovation Center. It replaces the voluntary Bundled Payments for Care Improvement Advanced (BPCI-A) and Comprehensive Care for Joint Replacement (CJR) models with a broader, higher-stakes mandatory framework.

1.1 The Five Surgical Episodes

Episode Category	Typical Post-Acute Path	Cost Variability
Lower Extremity Joint Replacement (LEJR)	SNF, Home Health, or Home	Moderate — well-studied from CJR
Surgical Hip Femur Fracture Treatment	SNF most common; high acuity	High — non-elective, unpredictable
Spinal Fusion	Home Health or SNF	High — wide cost range
Coronary Artery Bypass Graft (CABG)	SNF, IRF, or Home	Very High — new to bundling
Major Bowel Procedures	SNF or Home; variable LOS	Very High — new to bundling

1.2 How the Money Works

Under TEAM, hospitals continue to bill Medicare fee-for-service as usual. However, CMS retrospectively compares each hospital's actual episode spending against a regionally-benchmarked target price. The difference, adjusted by a Composite Quality Score (CQS), determines whether the hospital earns a reconciliation payment from CMS or owes CMS a repayment.

The financial risk escalates over the model's five years through three participation tracks:

- **Track 1 (PY1):** Upside only. Hospitals can earn shared savings but face no downside risk. Safety net hospitals may remain in Track 1 for up to three years.
- **Track 2 (PY2–5):** Two-sided risk with lower caps. Available to safety net, rural, Medicare-dependent, and sole community hospitals.
- **Track 3 (PY2–5):** Full two-sided risk with higher caps on both gains and losses. Default track for most hospitals from PY2 onward.

The Composite Quality Score adjusts reconciliation amounts based on performance across quality measures including hospital-wide readmissions, patient safety (PSI-90), THA/TKA patient-reported outcomes, and the Information Transfer PRO-PM (mandatory from 2027).

1.3 Why This Creates Urgent Software Demand

Three structural features of TEAM make software solutions essential for participating hospitals:

1. **Mandatory participation with escalating risk.** Unlike BPCI-A, hospitals cannot opt out. By PY2, most face real financial downside. The urgency to get episode management right is existential, not aspirational.
2. **Post-acute care is the largest cost lever.** Post-discharge spending (SNF, home health, rehab, readmissions) accounts for the majority of episode cost variability. Hospitals historically had zero visibility into this spending. TEAM forces them to care about what happens after discharge.
3. **Quality directly impacts financial outcomes.** A poor quality score can erase cost savings or amplify losses. Hospitals need integrated cost-and-quality tracking, not just one or the other.

2. Competitive Landscape

The TEAM vendor ecosystem is fragmented across five categories. No single company provides a complete hospital-side TEAM management solution. The following analysis maps existing players, their strengths, limitations, and strategic positioning.

2.1 Episode Analytics Platforms

Avant-garde Health

Founded / Funding	2014 / $12M Series A / 11–50 employees
Core Product	CareMeasurement — web-based surgical analytics platform integrating EMR, financial, and claims data with national Medicare FFS benchmarks
TEAM Play	Dedicated CMS TEAM product with Collaborative (40+ hospitals). Predictive modeling, physician-level scorecards, PAC utilization analysis
Strengths	Deep bundled payment expertise (HBS origin). Strong retrospective analytics. National benchmarking datasets. Established hospital relationships
Limitations	Primarily retrospective — analyzes past performance, not active episodes. No real-time discharge decision support. No care coordination workflow. Analytics-only (does not provide clinical services)

PurpleLab

Claims-based episode-of-care analytics platform with pre-built EOC system being adapted for TEAM. Broader real-world evidence platform. Less specialized in surgical episodes than Avant-garde but has faster implementation claims.

Innovaccer

Large value-based care platform (unicorn, $200M+ raised) with TEAM-specific marketing. Offers financial analytics, contract management, quality reporting, and care coordination tools. Strength is breadth across all VBC models. Limitation is lack of deep TEAM-specific focus — TEAM is one of many use cases, not their core product.

2.2 Care Coordination and Navigation Services

Deacon Health

Founded / Funding	2025 launch / $7M Seed / Nashville-based
Core Product	Tech-enabled nurse navigator service for MSK/specialty care. Licensed nurse navigators manage patients pre- and post-surgery. Shares TEAM downside risk with hospitals
Strengths	Risk-sharing aligns incentives. Proven 13.5% MLR reduction. Human-first approach with strong clinical credibility. Backed by major healthcare executives (HealthSpring, CHS, Ardent founders)

Limitations	Services-heavy model limits scalability. Not a pure SaaS product — requires deploying human navigators. Primarily MSK-focused (LEJR/spine). Limited coverage of CABG and bowel procedures

Signify Health (now CVS) and naviHealth (now Optum)

The two most experienced episode management platforms have been acquired by payer conglomerates. Signify Health managed $6.1 billion in BPCI-A spending and was acquired by CVS for approximately $8 billion in 2023. naviHealth, with its proprietary nH Predict assessment tool, was absorbed into Optum/UnitedHealth. Their episode platforms are now largely captive inside these organizations, making them unavailable to independent hospitals as standalone solutions. This acquisition pattern has created a significant vacuum in the market for independent, hospital-side episode management tools.

ThoroughCare

Generic care coordination software with TEAM marketing. Covers chronic care management, remote patient monitoring, and annual wellness visits. TEAM is a marketing overlay on existing capabilities rather than a purpose-built solution.

2.3 Care Transitions and Referral Management

WellSky / CarePort

Scale	2,500+ hospitals, 130,000+ PAC providers, 54 million referrals/year, 17 million discharges/year. PE-backed (TPG), 2,000+ employees
Core Product	Care transition platform connecting acute and post-acute providers. Referral management, discharge planning workflows, real-time patient tracking across care settings
TEAM Play	Announced enhanced TEAM capabilities in April 2025. Predictive analytics for discharge decisions (SNF vs. home health level-of-care prediction). New CarePort Referral Summary dashboards
Strengths	Dominant network effects. Largest acute-to-PAC transition platform in the US. AI-powered referral summaries. Deep data on PAC provider performance
Limitations	Primary revenue comes from PAC providers, not hospitals. Discharge prediction is not tied to TEAM episode economics. No episode-level financial tracking or target price comparison. Referral optimization serves the network, not the hospital's P&L

Profility and Clinware

Both sell to SNFs and PAC providers, not hospitals. Profility offers AI-powered referral intake and predictive patient placement, helping SNFs win hospital referrals. Clinware ($4.25M funding, 2025 launch) automates admissions and clinical workflows for SNFs. Both are potential integration partners for a hospital-side product but are not competitors.

2.4 Patient-Reported Outcomes (PRO) Platforms

TEAM's Composite Quality Score includes PRO measures, making PRO collection a compliance requirement. The market here is more mature:

- **CODE Technology:** Fully managed PRO collection service with industry-leading capture rates. Zero workflow disruption, 30-day implementation. Explicitly TEAM-ready. The strongest pure-play PRO vendor.
- **Force Therapeutics:** Broader digital care platform combining PRO collection with patient engagement, recovery pathways, and remote monitoring. Stronger on the patient engagement side than pure compliance.
- **PatientIQ / OBERD:** Cloud-based PRO collection platforms with registry integration. More research-oriented than operationally focused.

Assessment: PRO collection is a crowded, relatively mature segment. Building a standalone PRO product would mean competing against established, specialized vendors. However, PRO data is a valuable input to episode analytics and quality scoring — integrating with existing PRO platforms (particularly CODE or Force) is the smarter play.

3. Gap Analysis: Seven Opportunities

Based on our competitive research, we identified seven explicit gaps where no existing vendor adequately serves hospitals participating in TEAM. These gaps are ordered by estimated market impact and feasibility.

#	Gap	Impact	Feasibility	Competition
1	Real-time episode command center	Very High	Medium	None direct
2	TEAM-economics-aware discharge support	Very High	Medium	Partial (CarePort)
3	Non-ortho episode intelligence (CABG, bowel)	High	Medium-Hard	None
4	TEAM-specific PAC network builder	High	Medium	Weak (generic PAC data)
5	What-if financial simulator	High	High	None
6	Safety net hospital-specific tooling	Medium	Medium	None
7	Surgeon-facing episode performance	Medium	High	Partial (Avant-garde)

Green-highlighted rows indicate our recommended product focus. These gaps combine high market impact with viable feasibility and minimal direct competition. The following sections detail each gap.

3.1 Gap 1: Real-Time Episode Command Center

Avant-garde Health provides excellent retrospective analytics — looking backward at completed episodes to identify cost drivers and variation. WellSky tracks care transitions in real time. But no vendor provides a unified, real-time dashboard of active TEAM episodes that combines cost tracking against target prices, quality metric status, and care coordination milestones in a single view.

Why it matters: Hospital operations teams currently have no way to see, at any given moment, how many active episodes they have, which are trending over the target price, which patients are at risk for readmission, and what their projected reconciliation outcome looks like. They are flying blind until CMS performs reconciliation months after the performance year ends.

3.2 Gap 2: Discharge Decisions Tied to Episode Economics

WellSky/CarePort introduced predictive analytics in 2022 that assess appropriate post-acute level of care (SNF vs. home health) and readmission risk. However, this prediction is not connected to the hospital's TEAM target price for each episode.

Why it matters: The discharge decision is the single largest cost lever in a TEAM episode. The difference between a SNF discharge (~$12K–$22K post-acute cost) and a home health

discharge (~$3K–$6K) can determine whether an episode comes in above or below the target price. A tool that shows the discharge planner the financial impact of each pathway — in the context of that specific episode's economics — would be transformative.

3.3 Gap 3: Non-Orthopedic Episode Intelligence

Nearly every vendor in this space built their expertise on joint replacement episodes, the legacy of CJR and BPCI-A. TEAM introduces CABG and major bowel procedures, which have fundamentally different post-acute care patterns, complication profiles, and cost drivers. No vendor has deep analytics, care pathways, or benchmarking data for these non-orthopedic surgical categories.

3.4 Gap 4: TEAM-Specific PAC Network Builder

TEAM regulations encourage hospitals to form high-value post-acute care networks. CMS requires that SNFs receiving TEAM patients under the 3-day rule waiver have a minimum 3-star quality rating. Yet no product helps hospitals systematically build, score, and manage their preferred TEAM network — ranking SNFs and home health agencies by cost and quality performance per episode category.

3.5 Gap 5: What-If Financial Simulator

TEAM's reconciliation formula is complex: actual spending vs. target price, adjusted by a Composite Quality Score, with caps on gains and losses that vary by track. No existing product allows a hospital CFO to model scenarios such as: "If we shift 15% of SNF discharges to home health, reduce readmission rates by 2 points, and improve our CQS from 0.6 to 0.75, what is our projected reconciliation outcome?"

Why it matters: This is the CFO's entry point. A financial simulator is lower-complexity to build than a real-time command center, delivers immediate value during the planning phase (even before PY1 reconciliation), and creates a natural upsell path to operational products.

3.6 Gap 6: Safety Net Hospital Tooling

TEAM includes special provisions for safety net hospitals: extended Track 1 eligibility (3 years), Track 2 availability with lower risk caps, social risk adjustment in target pricing, and requirements for health equity plans and SDOH screening. Safety net hospitals face unique challenges including higher-acuity patients, social determinant barriers, and tighter resource constraints. No vendor offers purpose-built tooling for this segment.

3.7 Gap 7: Surgeon-Facing Episode Performance

Surgeon behavior (implant selection, length of stay decisions, discharge recommendations) drives a significant portion of episode cost variation. Avant-garde Health provides retrospective physician-level analytics, but surgeons have no real-time visibility into how their patients' episodes are performing financially or how their practice patterns compare to peers. Engaging surgeons is critical to TEAM success but is currently undersupported by available tools.

4. Recommended Product: TEAM Episode Navigator

Based on our gap analysis, we recommend building a three-module SaaS product that addresses Gaps 1, 2, and 5 — the highest-impact, most feasible opportunities with the least direct competition. We propose the working name "TEAM Episode Navigator."

4.1 Product Architecture: Three Modules

Module 1: Financial Simulator & Reconciliation Modeler

The door-opener. Sells to CFOs before operational products are needed.

Features

- **Target price calculator:** Ingests historical Medicare claims data and applies TEAM's regional pricing methodology, risk adjustment factors, and hospital-level adjustments to generate estimated target prices for each episode category.
- **Scenario modeling engine:** Interactive what-if simulator. Adjust discharge mix (% to SNF vs. home health vs. home), projected readmission rates, length of stay assumptions, and quality metric performance. See real-time impact on projected reconciliation payment or repayment amount.
- **Quality score impact analysis:** Models how changes to specific quality measures (HWR, PSI-90, PRO-PM) flow through the Composite Quality Score adjustment to affect financial outcomes.
- **Track comparison tool:** For hospitals eligible for multiple tracks, compares expected financial outcomes across Track 1, 2, and 3 to inform track selection decisions.
- **Benchmarking dashboard:** Anonymous peer comparison showing where the hospital's episode costs, discharge patterns, and quality metrics sit relative to regional and national averages.

Value by Persona

Persona	Value Delivered	Key Metric
CFO / VP Finance	Quantifies financial exposure before PY1 reconciliation. Enables proactive strategic decisions on track selection, PAC network investments, and care redesign priorities	Projected reconciliation $ per scenario
Chief Strategy Officer	Data-driven TEAM readiness assessment. Identifies highest-ROI improvement levers. Supports board-level presentations	Improvement opportunity $ quantified
VP Quality	Understands how quality performance translates to financial impact under TEAM's CQS	CQS sensitivity by measure

	adjustment. Prioritizes quality improvement initiatives	

Module 2: Episode Command Center

The operational engine. Real-time visibility into every active TEAM episode.

Features

- **Active episode dashboard:** Real-time view of all open TEAM episodes. Each episode shows: patient demographics, surgical category, current cost accumulation vs. target price, days remaining in the 30-day window, current care setting, and risk flags.
- **Cost trajectory alerts:** Automated alerts when an episode's spending trajectory will exceed the target price. Triggers at 50%, 75%, and 90% of target with recommended interventions.
- **Readmission risk scoring:** Predictive model that flags patients at high risk of readmission during the 30-day window. Incorporates clinical (comorbidities, surgical complexity), social (SDOH screens), and operational (discharge setting, provider quality) factors.
- **Quality metric tracker:** Rolling performance on TEAM quality measures. Shows how current episode outcomes (readmissions, complications, PRO submissions) are affecting the hospital's projected CQS.
- **PAC provider performance feed:** For patients discharged to SNFs or home health, shows the receiving provider's historical TEAM performance: average episode cost contribution, readmission rate, and length of stay.
- **Cohort analytics:** Aggregate views by episode category, by surgeon, by discharge destination, and by time period. Identifies patterns driving cost variation.

Value by Persona

Persona	Value Delivered	Key Metric
VP Care Management	Operational visibility into every active episode. Enables targeted interventions on high-cost episodes before they close. Reduces surprise reconciliation outcomes	% of episodes managed within target
Discharge Planners / Case Managers	Decision support at the bedside. See financial context for discharge decisions. Access to PAC provider performance data when making referrals	Discharge-to-home rate improvement
CMO / Chief Quality Officer	Real-time quality measure performance. Early warning on readmission trends. Connects clinical outcomes to financial impact	CQS trend and readmission rate

Module 3: Discharge Optimizer

The savings driver. TEAM-economics-aware discharge decision support at the bedside.

Features

- **Pathway recommender:** For each patient approaching discharge, predicts 30-day total episode cost and readmission probability for each post-acute pathway (SNF, IRF, home health, home self-care). Recommendations incorporate patient clinical data, SDOH factors, caregiver availability, and the hospital's current episode economics.
- **Financial impact overlay:** Shows the discharge planner exactly how each pathway option affects the episode's position vs. target price. Example: "Sending to SNF A projects the episode at $2,400 over target. Home health with Agency B projects $1,800 under target."
- **PAC network scorecard:** Ranks preferred PAC providers for each episode category. Scores based on: average cost contribution, readmission rate, patient satisfaction, LOS, and CMS star rating (for SNFs). Highlights which providers qualify under TEAM's 3-day rule waiver.
- **Patient-choice compliance:** TEAM regulations require that beneficiaries maintain freedom of provider choice. The tool presents recommendations to the care team while documenting that patient choice is respected — a compliance safeguard.
- **Surgeon alignment reports:** Shows each surgeon their patients' discharge patterns, episode costs, and outcomes vs. peers. Enables data-driven conversations between administration and surgical staff.

Value by Persona

Persona	Value Delivered	Key Metric
Discharge Planners	Data-driven discharge recommendations with financial context. Reduces guesswork. Ensures patient-choice compliance documentation	Average episode cost per discharge path
Surgeons	Transparency into how their practice patterns affect episode economics. Peer benchmarking motivates standardization without mandates	Surgeon-level cost per episode
CFO	Direct financial impact: shifting even 10% of SNF discharges to home health saves $8K–$16K per episode. Quantifiable ROI	$ saved per episode shifted

5. Go-to-Market Strategy

5.1 Target Market Segmentation

The initial target market is the approximately 740 hospitals mandated to participate in TEAM. We recommend segmenting this market into three tiers based on urgency and capacity to buy:

Tier	Profile	Est. Count	Priority
Tier 1	Large health systems (500+ beds) in Track 3 with high surgical volume. Highest financial exposure	~150–200 hospitals	Immediate — launch targets
Tier 2	Mid-size community hospitals (200–500 beds). Moderate volume, limited analytics infrastructure	~300–350 hospitals	High — fast followers
Tier 3	Safety net, rural, and sole community hospitals. Special track provisions, resource-constrained	~200–250 hospitals	Medium — future niche product

5.2 Sales Motion

We recommend a two-stage sales approach that leads with finance and expands to operations:

Stage 1: Land with the Financial Simulator (Module 1)

- **Buyer:** CFO or VP of Finance. These leaders own TEAM financial risk and are measured on reconciliation outcomes.
- **Entry point:** Offer a free TEAM financial exposure assessment using publicly available claims data and the hospital's historical episode volumes. This serves as a lead generation tool and proof-of-value demonstration.
- **Conversion trigger:** When the assessment reveals significant financial exposure (typically $2M–$10M+ at risk for large systems), convert to a paid subscription for the full simulator with scenario modeling.
- **Deal size:** $50K–$100K annual contract for the simulator module alone.

Stage 2: Expand with Operational Modules (Modules 2 + 3)

- **Buyer expansion:** After the CFO demonstrates value, introduce the Episode Command Center and Discharge Optimizer to the VP of Care Management, CMO, and VP of Quality.
- **Expansion trigger:** PY1 reconciliation results (expected mid-2027). Hospitals that underperform will urgently need operational tools. Hospitals that overperform will want to sustain gains.
- **Full platform deal:** $150K–$400K annual for the complete 3-module platform, depending on hospital size and episode volume.

5.3 Pricing Model

We recommend a hybrid pricing structure:

Component	Small Hospital	Mid-size Hospital	Large System
Module 1 (Simulator)	$50K/year	$75K/year	$100K/year
Module 2 (Command Center)	$75K/year	$125K/year	$200K/year
Module 3 (Discharge Opt.)	$50K/year	$100K/year	$150K/year
Full Platform	**$150K/year**	**$250K/year**	**$400K/year**

Alternative: Per-episode pricing ($75–$150 per TEAM episode) aligns cost with volume and is attractive to hospitals uncertain about adoption. However, it creates revenue volatility and is harder to forecast. Recommend offering as an option for Tier 2/3 hospitals.

5.4 Channel Strategy

- **Direct sales:** Primary motion for Tier 1. Hire 3–4 enterprise AEs with health system finance or VBC backgrounds. Target VP Finance and VP Care Management.
- **Consulting partnerships:** Partner with healthcare advisory firms (Vizient, Premier, Chartis Group) who are already advising hospitals on TEAM readiness. They generate leads; you provide the technology.
- **Conference-led demand gen:** TEAM is a hot topic at HFMA (healthcare finance), ACMA (case management), Becker's, and AAOS (orthopedics). Sponsor workshops, publish benchmarking data, and offer free exposure assessments.
- **Integration partnerships:** Integrate with Epic, Cerner/Oracle Health, and Meditech EHRs for data feeds. Build integrations with WellSky/CarePort for discharge workflow and CODE/Force for PRO data. These integrations reduce implementation friction and create distribution.

5.5 Key Messaging by Persona

Persona	Core Message	Proof Point
CFO	"Know your TEAM exposure before CMS tells you. Model the scenarios that turn risk into savings."	$ at risk quantified
VP Care Management	"See every active episode in real time. Intervene before costs spiral, not after reconciliation."	% episodes within target
Discharge Planners	"Make the right call at discharge with data, not guesswork. Know which providers deliver the best outcomes for your patients."	Readmission rate reduction

CMO / Surgeons	"Understand how surgical decisions flow through to episode economics. See your outcomes vs. peers."	Surgeon-level benchmarks

6. Phased Product Roadmap

Phase 1: Financial Simulator (Months 1–4)

Goal: Ship the door-opener product. Get into 10–15 hospitals with the financial modeling tool before PY1 reconciliation results arrive.

- Ingest historical Medicare claims data (CMS makes baseline data available to TEAM participants)
- Implement TEAM target price calculation methodology with regional benchmarks and risk adjustment
- Build scenario modeling engine with adjustable parameters (discharge mix, LOS, readmission rates, quality scores)
- Create track comparison tool and basic benchmarking dashboards
- Launch free TEAM Financial Exposure Assessment as lead generation

Phase 2: Episode Command Center (Months 5–9)

Goal: Add real-time operational capabilities. Requires EHR integration (FHIR/HL7 ADT feeds) and claims data pipelines.

- Build EHR integration layer (start with Epic via FHIR, Cerner/Oracle second)
- Develop active episode tracking with cost accumulation and target price comparison
- Implement readmission risk prediction model
- Add cost trajectory alerting and intervention recommendations
- Build quality metric tracking with CQS projection

Phase 3: Discharge Optimizer (Months 8–12)

Goal: Deliver the discharge decision support tool. Can overlap with Phase 2 development.

- Build pathway recommender using historical episode outcomes data and patient characteristics
- Implement financial impact overlay connecting pathway recommendations to episode economics
- Develop PAC network scorecard with provider-level performance metrics
- Create surgeon-level performance dashboards
- Add patient-choice compliance documentation workflows

Phase 4: Expand and Deepen (Months 12–18)

Goal: Extend the platform to cover all five episode categories and add advanced capabilities.

- Build CABG and major bowel procedure-specific analytics and care pathways (Gap 3)

- Add safety net hospital module with SDOH integration and health equity plan support (Gap 6)
- Integrate with WellSky/CarePort for referral workflow and CODE/Force for PRO data
- Launch self-service analytics for surgeons and department heads (Gap 7)
- Explore risk-sharing pricing models (gain-sharing on reconciliation savings)

7. Financial Opportunity Sizing

Conservative revenue projections assuming a phased rollout across the 740 TEAM hospitals:

Metric	Year 1	Year 2	Year 3	Year 4	Year 5	Notes
Hospital clients	15	50	120	200	300	~40% penetration
Avg. ACV	$75K	$150K	$225K	$275K	$300K	Module expansion
ARR	**$1.1M**	**$7.5M**	**$27M**	**$55M**	**$90M**	

These projections assume the product extends beyond TEAM's 2030 expiration as CMS is expected to launch successor bundled payment models. The analytics infrastructure and hospital relationships built during TEAM's 5-year window create durable competitive advantages.

8. Risks and Mitigations

Risk	Impact	Mitigation
CMS modifies or delays TEAM	Reduces urgency and market size	Build platform to support any episode-based model (BPCI successor, commercial bundles). TEAM is the catalyst, not the ceiling
EHR integration complexity	Slows Phase 2/3 deployment	Start with Epic (largest market share in TEAM hospitals). Use FHIR APIs for standardized data access. Offer manual data upload as interim
Avant-garde builds real-time	Direct competition on Gaps 1/2	Move fast. They are analytics-first; we are operations-first. Different DNA.

		Partnership or acquisition possible if threat materializes
WellSky adds TEAM financials	Bundling with existing platform	WellSky's revenue model (PAC-side) creates structural conflict. Hospital-side financial optimization is not their core. Monitor but don't over-index on this risk
Hospital budget constraints	Slow sales cycles	Lead with free exposure assessment. Per-episode pricing option. ROI payback within 5–10 episodes makes the business case compelling

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This document is confidential and intended for internal strategic planning purposes only. Market data and competitive assessments are based on publicly available information as of April 2026.